

Accessing Intimate Interactions: Investigating the need for More Inclusive Sex Education Resources

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ABSTRACT

Misconceptions surrounding disability and sexuality are still prevalent and people with disabilities are often depicted as asexual and incapable to lead fulfilling sex lives. As a result, many individuals with disabilities struggle to access adequate sex education with negative consequences such as unplanned pregnancies, body image issues and sexual exploitation. To explore needs and practices for accessing relevant and reliable information about disability and sexuality, we conducted semi-structured interviews with 5 participants. Results show that there are several topics which participants thought should be better explored in the context of disability and sexuality, but most available sex education resources are not inclusive. Moreover, people with disabilities, especially when young or less used to self-advocate for their own needs, face difficulties engaging in meaningful conversations around sexuality. Based on the results of our research, we make recommendations for areas where HCI research could significantly contribute to make intimate interactions more accessible.

CCS CONCEPTS

• Human-centered computing; • Accessibility; • Empirical studies in accessibility;

KEYWORDS

Disability, Sexuality, Sex Education, Intimacy

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1 INTRODUCTION

Sex and sexuality are often severely overlooked topics in much of the disability discourse [30]. For decades, silence on the topic was accompanied by oppression and brutal practices, such as forced sterilization, causing severe shame and trauma for many people with disabilities. As stated by Shakespeare 2000, “*Sexuality, for*

disabled people, has been an area of distress, and exclusion, and self-doubt for so long, that it was sometimes easier not to consider it, than to engage with everything from which so many were excluded.” [31]. In more recent years, scholars and activists have been strongly arguing for more research on the topic and for a change in discourse towards sexual liberation for people with disabilities [18]. However, firsthand accounts of how people with disabilities negotiate access to sex, intimacy and relationships, and the barriers they face are still lacking [32].

Previous research has observed that amongst youth with disabilities, similarly to their peers without disabilities, many leverage the internet to form relationships, express their sexualities, consume sexual content and access information [8]. Nonetheless, it is unclear what other strategies and sources of information about sex and sexuality were used by young people beyond the internet and various social media.

In this paper, we present the results of a semi-structured interview study involving five participants with different disabilities that aim to understand how they accessed relevant information about sex and sexuality (both through digital and non-digital means), which sex education topics they thought were most important for people with disabilities, which ones were more often overlooked, and what they identified as the most relevant barriers to more inclusive sex education. Based on our results we present a series of recommendations for how future HCI research could contribute to the development of more inclusive resources for sex education that could address the needs of people with disabilities.

2 RELATED WORK

Previous research investigating the perceptions of different groups, from healthcare providers and the general public to people with disabilities themselves, has uncovered that many individuals with disabilities are often victim of incorrect stereotypes and stigmatization that portrays them as asexual, infantilised and unable to lead full and satisfactory sex lives, particularly with a partner who does not have a disability [12]. This aligns with the theory of sexual script that describes sexuality as a social process shaped by one’s internalized belief of how sexual behaviors should be played out, often following highly stereotypical genderized norms [5]. These external misconceptions are extremely damaging, affecting both physical and mental well-being of people with disabilities [28]. Sexual stigma may affect confidence, desire, and one’s ability to find a partner. At the same time, these social and cultural barriers can lead to internalized misrepresentations ranging from confusions about one’s sexual identity, reduced-self-esteem and doubts about asexuality [28].

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Many of these stereotypes are perpetrated by ableist public imagery and a severe lack of sex education resources that touch upon the topic of disability or, even more rarely, feature inclusive material about disability and sexuality [4, 5]. As a result, people with disabilities often struggle to access comprehensive and relevant information about sex and sexuality, which could empower them to live more fulfilling, safe and pleasurable sex lives [1, 9]. Coupled with the reduced independence experienced by many people with disabilities, this lack of knowledge increases the risks of suffering from violence and abuse and other potentially harmful sexual consequences, including unplanned pregnancies and sexually transmitted diseases [13, 17, 25, 29].

Another factor that make it significantly more difficult for people with disabilities, in particular young people with disabilities, to engage in conversations about sex and sexuality, is that parents, educators, and even health professionals, often feel uncomfortable or unprepared to discuss sexual issues or provide advice [9]. Findings from East & Orchards interviews with young people with physical disabilities, their parents and health professionals reveal a complex web of embarrassment and unpreparedness which results in a constant struggle to “*push away the issue*” from both parents and healthcare professionals. Common excuses used to justify this behavior were arguing that it was either not yet the time for discussions about sex and sexuality, or that such a discussion should be led by someone with more experience than themselves. The result was that young people would rarely receive comprehensive information about sex [9].

Over the years, both researchers and practitioners have developed and deployed a number of interventions to promote better sexual education amongst people with disabilities [26, 27, 33]. For example, a computer-based interactive multimedia program that aimed to reduce risk of HIV transmission amongst women with intellectual disability was shown to be effective in increasing knowledge of HIV, willingness to use condom, and engage in safe sex related behaviors [33]. Wells et al also observed that the use of interactive technology increased the accessibility of the material for participants who had limited literacy [33].

However, reviews exploring the content of various interventions delivering sexual education to people with disabilities highlight that the majority of the topics centered around health aspects, self-awareness, safety, and biological matters (such as puberty and menstruation), largely overlooking other topics that were important to users such as information about pleasure, relationships, and personal sexuality [23, 26]. Conversely, the discourse analysis carried out by Hall 2018 [14], who examined the conversation amongst members of a disability specific forum, reveals that topics that drew most engagement from individuals ranged from practical matters such as advice on experimenting with different sexual positions to vulnerable conversations around relationships, pleasure, desire and intimacy.

Ultimately, as highlighted by Schaafsma et al. 2017 [25], regardless of the medium used to deliver sex education programmes and interventions, people with disabilities were rarely involved in their development. To this end, it is important to explore how individual with disabilities currently access information about sex and sexuality, what information they deem important, and how they think sex educational content could be made accessible more effectively.

3 METHODS

3.1 Interviews

From the brief literature review presented in the previous section, it is evident that there are many significant barriers that hinder the ability of people with disabilities to access sex education content and information that might empower them to achieve a more fulfilling sexual life. To gain a better understanding of which information about sex and sexuality people with disabilities are interested in acquiring and what are the current difficulties that they encounter in accessing them, we conducted semi-structured interviews with five participants [P1 - P5]. These five participants reported different ages, backgrounds and disabilities as referenced in Table 1. Two participants were from the UK, one from the US, one from Canada and one from South Africa. Self-reported age ranged between 23 and 48. Two participants identified as female, two as male and one as non-binary.

Participants were recruited through social media, relevant mailing lists and forums targeting people with disabilities. Inclusion criteria for the study were age above 18, self-identify as a person with disability, and ability to provide informed consent to take part in the study. As the study was conducted during the COVID-19 pandemic, all interviews were carried out remotely using the video-conferencing software Microsoft Teams. This platform was preferred to other alternatives as it offers enhanced security and better data protection options compared to other similar software. After the conclusion of the data collection, participants were entered in a draw for two 25£ vouchers to thank them for their participation.

The interview was structured around three main subsections. In the initial part of the interview participants were asked about their knowledge and opinion of current practices in the delivery of sex education, what they considered reliable sources of information already available across different domains and what mechanisms could be used to access them, who they thought people with disabilities were more likely to approach if they had questions related to their own sex lives, and what barriers and facilitators might affect people with disabilities' ability to obtain reliable information concerning sex and sexuality. In the second part of the interview, participants were asked their opinion about the importance of including a variety of topics into what could become a “blueprint for an inclusive sex education programme”. These topics included contraception, sexually health, masturbation, puberty and sexuality, pregnancy and conception, inclusive sex toys, consent and communication. Participants were also asked to suggest any other topic that they deemed important. Finally, in the third part of the interview, participants were asked about their opinion on what role technology is currently playing in access to sex related information, how could it be better leveraged to improve access to inclusive information about sex and sexuality, what advantages could offer over non-technological means, and what limitations and safeguarding mechanisms would need to be considered.

It is important to notice that to protect participants' privacy and to avoid the disclosure of potentially sensitive information, participants were never asked about their personal experiences in relation to sex and sexuality, but rather their expert opinions on what knowledge and information about sex and sexuality are important for people with disabilities in general.

Table 1: Overview of participants' characteristics

Participant ID	Age	Country	Self-Identified Gender	Self-Identified Type of Disability
1	38	UK	Male	Physical Disability
2	48	South Africa	Male	Physical Disability
3	41	UK	Non-binary	Physical Disability
4	23	US	Female	Physical Disability
5	25	Canada	Female	ADHD

The length of the interviews varied between 35 and 70 minutes. Interviews were audio recorded and transcribed verbatim. Transcripts were analyzed inductively using thematic analysis to elicit emergent themes [3]. Both authors conducted the initial coding separately using the NVivo 12 Software, codes were discussed by the two authors and themes were conceptualized through consensus.

4 RESULTS

Three main themes were identified from the interview data: “*The pitfalls of prejudice, shame and ignorance*”, “*The importance of having someone to talk to*”, “*No topic is off the table*”, we unpack each of them in the sub-sections below.

4.1 The pitfalls of prejudice, shame and ignorance

All participants stated that access to sex education programs and/or relevant information about sex and sexuality is severely limited for most people with disabilities. Only P3 was aware of dedicated sex education been provided by occupational therapists to individuals who had a Spinal Cord Injury (SCI), as part of comprehensive rehabilitation programmes in the UK. All other participants were not aware of sex education programmes, inclusive or specifically targeted to people with disabilities, that were delivered by any “official channels”, such as government national health systems, charities or within schools. Participants agreed that one of the main root causes of this shortcoming is that people with disabilities are seen by most as asexual.

“It’s the stigma of disability. They think disabled people cannot be found sexual or beautiful at all because they are disabled.” – P4

“We are just seen as children. Yeah. . . I mean it is either children or inspirations, you know. We are never seen as just people.” – P3

When referring to the possibility of a sex education class being offered in a mainstream school, which all but one described as an unlikely event in most schools, participants were adamant that any pupil with disability would be automatically excluded from it.

“Sex education is mostly ignored in school anyway. But if it wasn’t a disabled person would be excluded from the class because it’s not for them, or they would be in the class, but they wouldn’t have anything directed at them you know” – P5

Even if one was to proactively seek out information about disability and sexuality, this would likely be a difficult and lonely quest. Many young people with disabilities are often surrounded by peers,

teachers, friends and family members who do not have a disability. Most of the people within their social circles lack the relevant knowledge to provide factual information, have little shared experiences and, especially in the case of parents, feel embarrassed and ashamed to even begin conversations about what is considered a “taboo topic”.

“Being born disabled in non-disabled family, you are just an outsider. And there is also no sharing tips and tricks and tools in schools if you go to mainstream.” – P1

Shame and embarrassment were not seen as being exclusive to parents or other family members without disabilities. Participants believed that many individuals with disabilities themselves might feel uncomfortable about attending a sex education course, and potentially be reluctant to take part even if the opportunity was offered to them.

“I am not sure if many people with disabilities I know would feel ok going to a sex education class in person. I think they would be afraid of meeting local witnesses. . . People that they know. You don’t want to let other know that not everything is ok in that area” – P2

4.2 The importance of having someone to talk to

Participants felt that most people with disabilities were left alone in their quest to obtain information about sex and sexuality. P4 and P1 believed that many people with disabilities might benefit from being able to receive support from a therapist or a counselor with the right kind of expertise. However, P5 highlighted how access to these services can be quite expensive, making it an unfeasible option for most people with disabilities. This will be especially challenging for those who are young and might be financially depend on their parents, or simply lack a stable source of income. Moreover, P2 and P3 stated that therapists or counselors with relevant expertise, and who have an inclusive outlook towards sexuality, can be extremely rare and difficult to find. Finally, the prominent stigma surrounding disability and sexuality in many countries might make individuals with disabilities reluctant to approach an unknown professional for fear of being dismissed or shamed.

“I feel like there is a lot of barrier towards the way that professionals see disabled people and treat disabled people don’t have that sort of relationship or communication where people might feel comfortable

talking about that in this country. I know in other countries it's a lot different." – P3

As parents, teachers, and even qualified professionals were not seen as a potential point of contact for accessing information about sex, participants felt that most people with disabilities were likely to rely on friends, especially the ones who were also persons with disabilities, to ask questions or simply discuss sexuality related topics that were important to them. Nonetheless, participants stated that many people with disabilities might not have friends with similar experiences in their immediate circles, hence they would be likely to rely on virtual communities or forums.

"I think in terms of friends, it's difficult for people with disabilities because you might approach them with questions because friends are more likely to talk about sex. But if they don't share the same disability as you, then they might not have the information that you need for it. I would say, the first approach would probably be online, like Googling things and asking in the community online." – P5

Overall, participants described online resources as extremely useful, and often downright essential for most people with disabilities. Forums, blogs, online videos and other platforms didn't just enable people to access information about sex that would otherwise be difficult to find, but also allowed people to ask questions anonymously when they might be too embarrassed to ask them in person.

"Yeah, medical staff or other people can judge you, but computers can't judge you" – P4

At the same time, participants stated that many of the resources which are available online are of poor quality and might contain incorrect information that might be misleading and harmful for the individual. For example, P1 highlighted how many young people might gain sexual knowledge through pornography. While on the one hand, pornography featuring people with disabilities can be seen as liberating as it helps to shatter incorrect beliefs, it could also be exploitative, extremely damaging for mental health, and promoting unrealistic ideals of sexual performance. To help people filter content, learn to evaluate information, and obtain personalized support when needed P2 suggested that a blended approach could provide the best solutions for many individuals with disabilities.

"I think that education should not only be digital, it should also be face to face with maybe health practitioners, doctors, teachers, and other persons with disabilities as well! – P2

4.3 There shouldn't be any topic which is off the table

As part of the interview, we asked participants to mention any topic or area of interest that an inclusive sex education programme should feature. Their response was extremely varied and highlighted the need for tailored approaches to provide information concerning sex and sexuality in relation to topics tackled, disability-specific facts and means of delivering the actual content. Some topics, such as contraception, prevention of sexually transmitted diseases, and

masturbation were seen as universally important regardless of the age, disability, gender and sexual orientation of the individual.

"Teaching about masturbation to all is really important. It's very important to recognize that people have different types of bodies and different ways that they can use their bodies and like different methods that they might explore their bodies with. – P5

"Contraception is crucially important. Having a child accidentally is awful, which still happen today in England for people with disabilities, because they have not been told about it" – P1

Other topics were seen as being more specific to people with different characteristics. For example, matters such as puberty and aging in relation to sex and disability could be tailored to people depending on their age. Similarly, specific information about pregnancy, conception and genital hygiene care might be relevant to people with conditions that present particular challenges. Conversely, other topics might be relevant depending on the sexual preferences of the individual such as information pertaining BDSM practices or the use of sex toys.

"Pregnancy is a strange thing, both a risk and a very good thing, depending on where you are in life. It is also important to know the risk to have a child and responsibilities and how conception works for someone with a physical disability" – P3

"Yes, this one [genital hygiene care] is very important because you also have to know about a catheter, and you have to be very careful when it comes to yeast infections because you can get those from a sexual activities and you don't want that to interfere with your catheter if you have to have one. – P4

Another topic that all participants felt is currently largely overlooked by most sex education resources, but is extremely important to people with disabilities, was related to communication and consent. Firstly, this was considered essential as participants felt that people with disabilities might often be more vulnerable to sexual exploitation. Secondly, many participants with disabilities might have sexual relationships with someone who does not have a disability or has a different disability. This might lead to different needs and desires that one should be able to communicate with their partner.

"Consent is an interesting thing because a lot of disabled people get touched personally by carers or others that are there to help. But that can lead you know that can lead very easily to abuse. Yeah, and then we need to give disabled people the tools to understand the good from the bad, the right from the wrong and give them the tools to communicate when some when they do feel that something is wrong or off or whatever and you know to make them feel comfortable." – P3

Finally, participants also highlighted how, although people with different types of disabilities might have specific needs and interests that need to be addressed in a tailored manner, more general topics of sex education that are relevant to all should be provided in an

inclusive manner to help reduce misconceptions and favor a shared understanding between people with and without disabilities.

“I would love for sex education to be available for all people with all disabilities, as inclusive as it can be. But that it doesn’t exclude people without disabilities from looking at it. Because I’m very. . . I really like creating sex education material that applies to everybody, but that everybody needs to see. . . Like even thinking back to grade school when they would split up the boys and the girls and they would teach the girls about periods and men have no idea how a period works. . .”

– P5

5 DISCUSSION

The findings from this study highlight some of the key challenges faced by people with disabilities in accessing sex education and obtaining relevant information related to sex and sexuality. The study also helps to uncover current practices used by people with disabilities to independently acquire sexual knowledge and address personal queries. Finally, the results identify areas of interests for inclusive sex education resources and suggests strategies for more effective delivery of content.

Many of the challenges mentioned by participants in our study resonates with what has been reported in the literature [19]. The infantilization of people with disabilities that was often mentioned by participants, especially common amongst parents, teachers and careers is in line with what reported by Santos & Santos 2018 [24] and Carter et al 2021 [6]. As shown by Liddiard and Slater 2018 [16], participants in our study also hinted at how the normative views of beauty and sexuality causes the systemic exclusion of disabled bodies from mainstream sex related media, from educational content to pornography, which trickles down to individual views of who can, and cannot be, sexual amongst professionals, parents, potential partners and people with disabilities themselves.

Previous scholars, including Ebrahim 2019 [11], have made the case that technology in the form web-based resources that promote more diverse and uncensored representation of sex and disability, including disability specific pornography, can help to shatter pre-conception and have a significantly liberating effect. P1 expressed a somehow similar view, highlighting both positive and negative aspects of pornographic material featuring people with disability, which include the risks of othering of disabled bodies and the exploitation of actors with disabilities. Moreover, P1 pointed out how pornographic content can often create unrealistic ideals of sexual performance amongst young people with disabilities as seen for their peers without disabilities [15]. These tensions highlight how technology might work as a gateway to access more diverse and inclusive representation of sexuality that can help bring down barriers that are ableist, heteronormative and patriarchal [24]. However, there needs to be significant attention dedicated to ensuring that technology isn’t merely leveraged to promote exploitative content or facilitate abusive practices [10, 21].

As a person with disability might struggle to find someone with whom they can share experiences and have honest conversations about sex within their immediate social circles, our findings also suggest that technology could help to facilitate these conversations

amongst people with disabilities through dedicated groups [20]. Furthermore, technology can also support people with disabilities in gaining access to professionals with specific expertise, who might not otherwise be available within their own area. One interesting aspect of technology mediated communication that participants identified, is the perceived layer of anonymity that it offers to users, which might make one more likely to ask questions without feeling shame or embarrassment. However, it is essential for any community where members share questions and experiences about sex to be a safe space from predatory or abusive behaviors of any kind, but also secure from any potential inappropriate or exploitative use of highly sensitive data. Previous studies from Bennett 2017 [2] and Porter et al 2017 [22], highlighted some of the practices used by individuals with disabilities to shield themselves from unwanted contact when it came to dating websites and applications, but it is unclear if people with disabilities use similar strategies in online communities where they might exchange information about their own sexual lives.

Participants also highlighted the need for technology to help identify and filter resources with information about disability and sex according to one’s needs and interests. From universally relevant content that should be promoted for all to facilitate inclusive sex education, to specific topics of interest that might be key for some individuals but less important to others, technology should enable users to identify the resources that match their need at a particular time. For these information to be truly accessible, it is paramount to ensure that valuable resources that can facilitate inclusive sex education are made available in a variety of formats, to address the needs of individuals with different capabilities and learning styles.

Finally, researchers and designers should strive to better understand how effective existing and future technologies are in improving one’s sexual well-being. Previous research by Cormier & O’Sullivan 2021[7], showed that almost a third of young adults access sex technologies with the goal of improving their own sex lives. However, access to sex technologies was linked with overall lower self-reported sexual functioning. Ultimately, although technology might have the potential to make sex education more accessible, more effort needs to be invested in understanding its role and finding optimal ways to ensure it promotes the sexual wellbeing of all users in an inclusive manner.

6 CONCLUSIONS AND FUTURE WORK

This paper helps to further our understanding of the barriers that people with disabilities face when seeking to acquire relevant knowledge and information about sex and sexuality, uncovering existing practices, and highlighting needs and possibilities for the development of more inclusive sex education resources. The results from the interview study points at how many of the existing barriers are linked to incorrect representation of people with disabilities which perpetrates ignorance, prejudice and stigma, including self-stigma. Participants stated how it is important for people with disabilities to be able to ask questions and share experiences with others, but pointed out to the difficulties of doing so, as many within their immediate social circles might be unable to provide support. Finally, when discussing topics which they considered important

for the development of more inclusive sex education resources, participants pointed out how some topics should be seen as universally relevant also for people without disabilities as they would help to dispel misconceptions and promote a shared understanding. Through our reflections we highlight some key implication for how technology could become a vital tool to facilitate better access to information for individual with disabilities and promote the liberation of sexual content from dominant ableist narrative. However, we suggest that there is a need to remain mindful of potential counterproductive effects of technology being used consciously or unconsciously to reinforce stereotypes or facilitate abusive practices. To further investigate how technology could be leveraged to facilitate access to information about sex and sexuality for persons with disabilities, we plan to conduct co-design workshops with participants with disabilities to develop novel ideas supporting the delivery of inclusive sex education.

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